

اسم المريض رابعياً : تھانی ثروت ناشد عجمی

المحافظة : القاهرة

الحي / المدينة : الساحل

العنوان : ---

رقم البطاقة : 27204190103427 رقم التليفون : 01202020672

تاريخ تسجيل الإستمارة : 26-09-2026

العمر (تاريخ الميلاد) : 19-04-1972 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) المقرر بما فيه
التوقيع: (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادة سرطان الثدي علاج تدعيمى وتلطيفى [6579] **قسم العلاج :** الاورام

----- التعليق على التشخيص :

الاسم: د/محمد كيلاي
 الاسم: د/ندي حسن
 الاسم: د/نرمين رمضان
 الوظيفة: _____
 الوظيفة: _____
 الوظيفة: _____
 تعتمد صحة البيانات المرفقة
 رئيس اللجنة ومدير
 المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

-----/d -----/d -----/d.l -----/d.l

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

رئيس

المجلس الطبي المتخصص

مدیر عام

مديرية الشئون الصحية

ختم

شعار الجمهورية

Name: نهاني ثروت ناشد عجم
National ID: 27204190103427



تقرير طبي

السيد

تحية طيبة وبعد,,

بشان تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الطول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالبدی و تحتاج الي Supportive and palliative Treatment

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



جمهورية مصر العربية
مملكة البحرين الشخصية

تهاني
ثروت ناشد عجمي

٦ سن عمر مكرم - حتى الزفاف
العمرانية - الجيزة

١٩٧٢/٠٩

٢٧٢٠٤١٩٠١٠٣٤٢٧

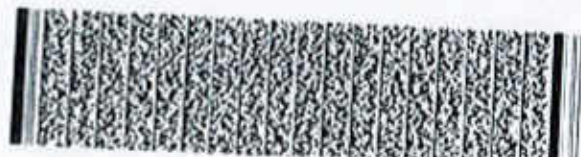
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٢٠٢٤/٠٦ ٢٧٢٠٤١٩٠١٠٣٤٢٧
حاصلة على بكالوريوس علوم حاسب الى

أنثى مسيحية
هاني حنا شاكر عطالله

البطاقة مصرية حتى ٢٠٣١/٠٥/٢١



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

03/26/2026

MDT FORM

Name:	Tahani Tharwat Nashed Agamy	MRN:	9593
Age:	54 Y	ID:	27204190103427
Menopausal Sta	post	Phone:	01202020627
NRS SCORE:		Marital Status:	married
BMI:		Family History:	-ve
CO Morbidity:	HTN-Hypothyroid	Reason Of MDT:	1st line MBC

Clinical Data:

BX:	IDC						
ER:	0/8	PR:	0/8	HER2:	3+	KI67:	

R

L

Sonomammography:

Mets Workup:

History:

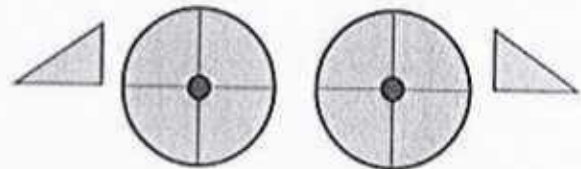
Diagnosed case of Lt breast IDC cT3 N3 ER0/8 PR0/8 Her2 3+ in 2024...pt received NACT TCHPx6...Lt MRM in 11/2024 revealing ypT1 N1 disease...PORT & Kadcyca x 12 ending in 8/2025...kept under follow-up....In 12/2025, PET/CT scan revealed brain SOL...pt received WBI 10 sessions on private basis ending 1/2026...and started Tykerb/Xeloda with PR (MRI 3/2026)

Recommendations:

Consider resuming Tykerb/Xeloda

MDT Outcome & Plan

for Tykerb/Xeloda



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ شيماء لاشين / د/ لذي حسن	د/ خالد السجدي				



Patient Name : الأستاذة / نهدي ثروت محمد

Patient No. : 27204190103427

Path No. : 4271/24

Sex : Female

Age : 52 Y

Referring Doc :

Request No. : 13004456

Request Date : 05/11/2024

Verification Date : 12/11/2024

Financial Category : نفقة الدولة - دور رعاية المسنين

Department : Out Patient

Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer post NACT

Nature of the specimen:

Left wide local excision and axillary clearance specimens , collected at 5/11/2024.

Gross Examination:

Three specimens were recieved :

-Left wide local excision specimen measured 9x8.5x2.5 cm with no covering skin ellipse .

• Tumor extent

o Number: An ill defined rubbery to firm greyish white tumor bed site .

o Size: 3.5x2.5x1.5 cm.

o Skin involvement: Free grossly

• Distance from margins: 0.1 cm from least superior margin .

-Axillary clearance specimen : Fibrofatty tissue piece measured 8x4.5x2.5 cm , enclosing 12 nodules up to 2.5x1 cm.

-Extra lateral margin: A fibrofatty tissue piece measured 13.5x9x2 cm with covering skin ellipse measured 8x2.5 cm, it showed gross indurations.

• Frozen section:

-Lateral and superficial margins are involved by DCIS.

-Extra lateral margin is involved by DCIS and suspicious for invasive carcinoma.

-Free other margins.

Microscopic examination:

• Histological type: Residual Invasive duct carcinoma.

• Grade (differentiation + nuclear pleomorphism + mitosis): G II(3+2+1).

-Tumor cellularity: 10% (0.2 cm in maximal dimension).

• Angiolinvasion: No.

• Ductal carcinoma in situ: Focal (9%).

Verified by :

DR.Menna Nabil

Printing Date : 16/12/2024 11:18:55

Page 1 of 2

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - اورشليم النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mar El Qadima - Cairo, Egypt



CamScanner



Patient Name	: الأستاذة / نهال ثروت شعبي	Request No.	: 13004456
Patient No.	: 27204190103427	Request Date	: 05/11/2024
Path No.	: 4271/24	Verification Date	: 12/11/2024
Sex	: Female	Financial Category	: اورام نثر المسام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

- Lymph node involvement: Positive (3/12).
- Resection Margins: Free all resected margins
- Histological response to therapy: Pathologic partial response.

Diagnosis:

Left breast cancer post NACT, wide local excision and axillary clearance specimens:

-Residual invasive duct carcinoma GII with focal DCIS.

Maximal invasive tumour diameter 0.2 cm.

-Tumor cellularity: 1%.

-Resection margins:

Extra lateral margin showed DCIS. Free of invasive tumour.

Free other resected margins.

-Lymph nodes :Positive (3/12).

-Pathologic partial response.

-Stage : ypT1a , ypN1.

- Residual cancer burden: 2.414, RCB class II.

Prof.Gamal Abdelatty

Dr.Nermine Muhammed

Verified by :

DR.Menna Nabil

Printing Date : 16/12/2024 11:18:55

Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche- Misr El Qadima - Cairo, Egypt



CamScanner

9047

Name: Tahany Tharwat Nashed
Referred by: Prof. Wael Samir Makkar

ID: 3370768
Date: 15/12/2025

¹⁸F-FDG PET/CT examination

Clinical Indication:

- Patient with history of right cerebellar lesion, and left breast cancer, underwent left mastectomy followed by chemoradiotherapy and hormonal therapy, for assessment of response.

Administered activity: 329 MBq. ¹⁸F-FDG

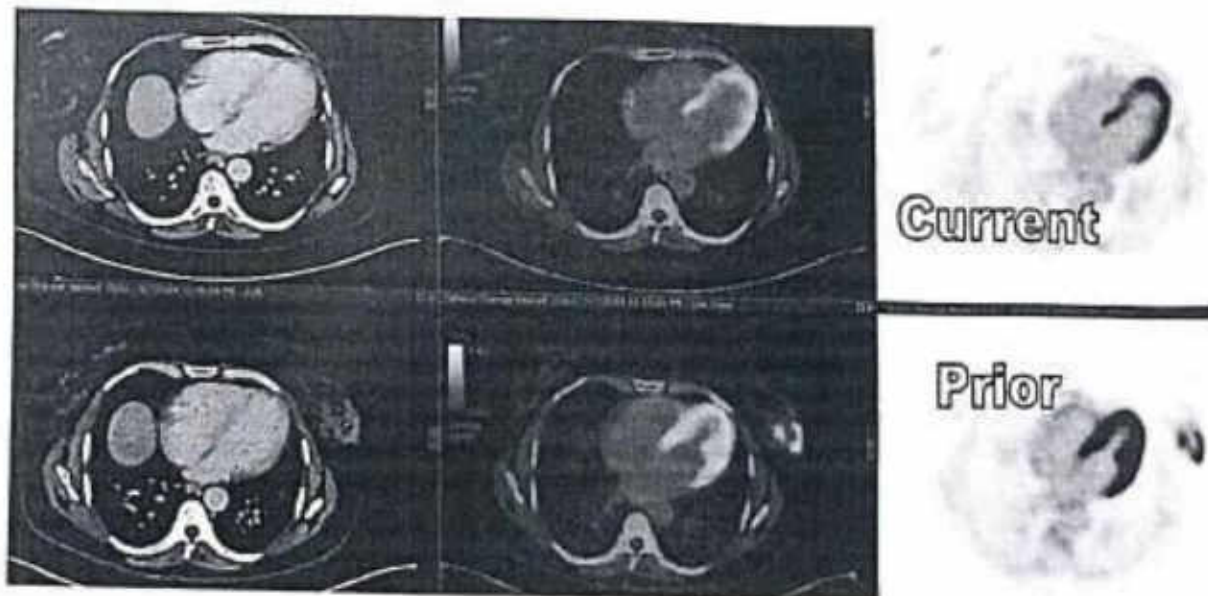
B.G.L. : 118 mg/dL

Examination protocol:

- A Whole body PET/CT study was obtained from skull vertex to the mid-thigh using a low dose non contrast CT for attenuation correction
- Diagnostic multislice CT examination after IV contrast administration of the same region.

Comparison was made to the FDG PET/CT study dated 07/05/2024

1. PET/CT Findings:





S C A N

Name: Tahany Tharwat Nashed
Referred by: Prof. Wael Samir Makkar

ID: 3370768
Date: 15/12/2025

¹⁸F-FDG PET/CT examination

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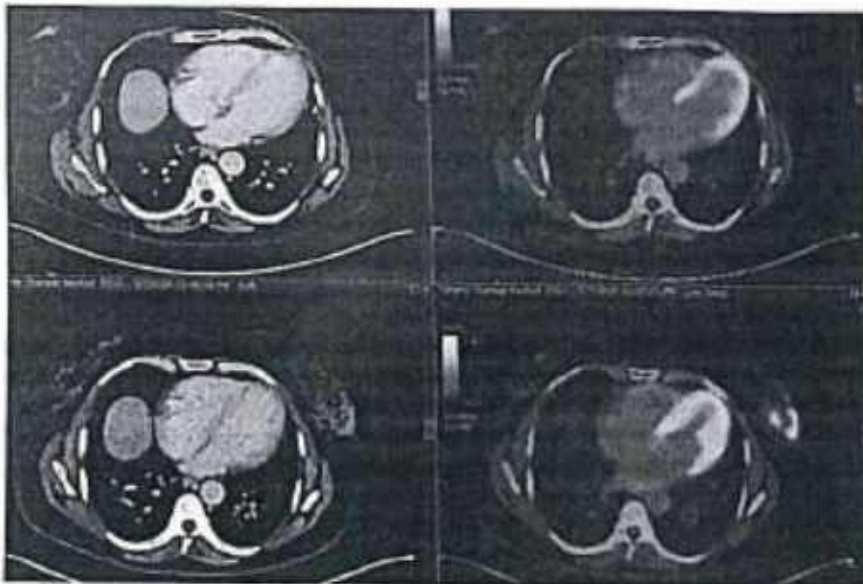
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Comparison was made to the FDG PET/CT study dated 07/05/2024

1. PET/CT Findings:



Current

Prior

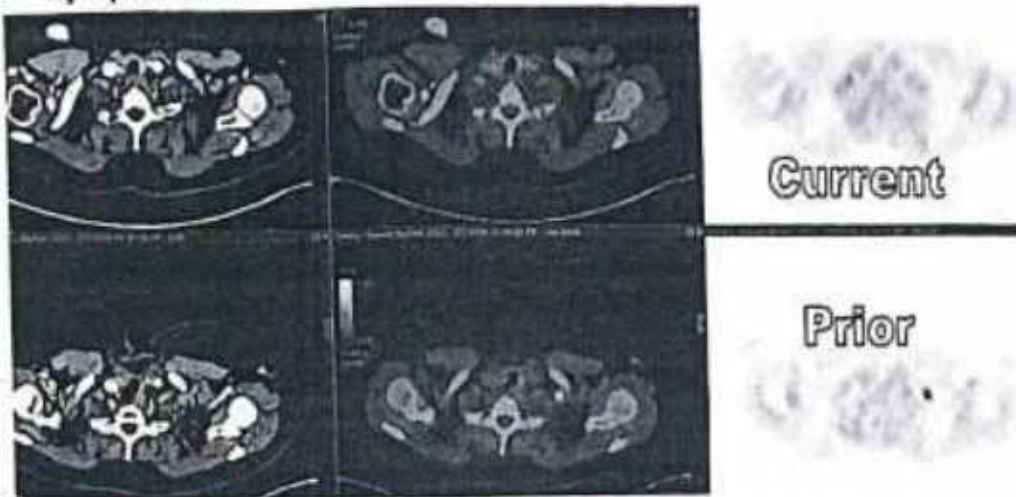
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A MEMBER OF ALFA MEDICAL GROUP

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الفاسكان

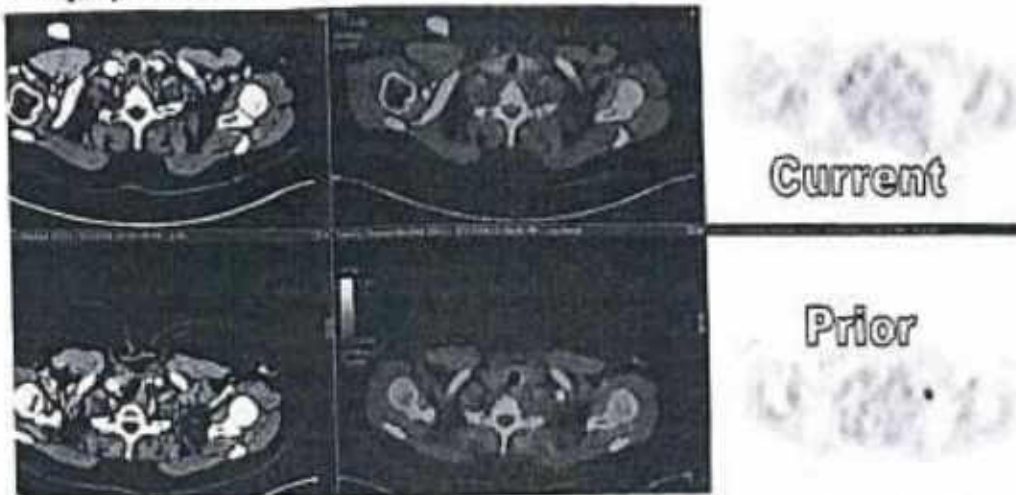
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- Interval left mastectomy and axillary dissection bed with low grade FDG uptake within soft tissue thickening, likely post-operative sequelae. No FDG avid lesions.
- Lymph nodes:



- Almost metabolic resolution of the previously described:
 - Cervical, left infraclavicular lymph node.
 - Mediastinal, left internal mammary lymph node.
- No newly developed FDG avid or pathologically enlarged lymph nodes.
- Brain:
 - Newly developed marginally enhancing FDG avid left occipital lesion with SUVmax 12.2 and measures 3.3x 1.6 cm associated with hypometabolism of adjacent parenchyma related to vasogenic edema and effacement of adjacent sulci.

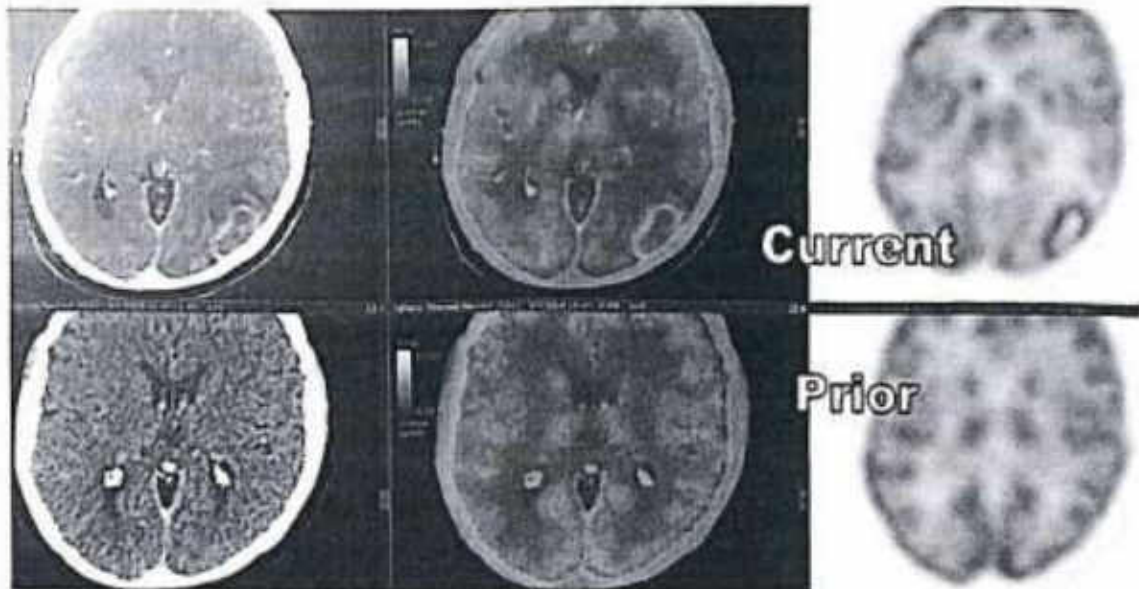
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- Resolution of the previously noted FDG activity within hyperdense hemorrhagic lesion in the medial aspect of the right cerebellum.
- **Right breast, no FDG avid lesions.**
- **Thyroidectomy operative bed, no FDG avid lesions.**
- **Lungs:**
 - Still noted few tiny bilateral pulmonary nodules with no significant FDG uptake.
 - Still noted non-FDG avid calcified middle lobe granuloma.
 - **Currently noted low grade FDG uptake within left peripheral atelectatic changes, *post-radiation sequelae*.**
- **Liver:**
 - Average sized, with no FDG avid focal lesions. No dilated biliary radicles. Patent main portal vein.
 - Still noted non-FDG avid hypodense segment II lesion, measures about 1.2 cm, *likely cyst*.

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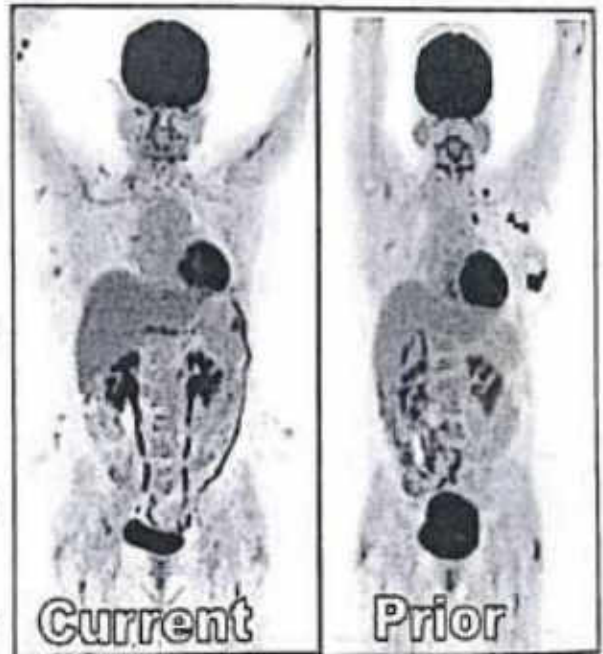
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- Spleen, no FDG avid focal lesions.
- Pancreas, no FDG avid focal lesions.
- No FDG avid lesions or abnormal mural thickening seen within small or large bowel.
- Stomach, no FDG avid lesions.
- Pelvis: No FDG avid lesions.
- Bone marrow, no FDG avid focal lesions.
- FDG activity related to bilateral gluteal injection sites.

II. Diagnostic CT Findings:

- No pleural, pericardial or peritoneal collections.
- The adrenals, kidneys (left renal cyst), aorta and IVC appear unremarkable.
- Right port-catheter.



OPINION:

Follow up study after interval left mastectomy and axillary dissection demonstrates:

- Low grade metabolically active soft tissue thickening within left mastectomy and axillary dissection bed, post-operative changes.
- Newly developed metabolically active left occipital brain lesion as described likely metastatic for MRI.

Radiologist
Dr. Mohamed Hosony, MD

Nuclear Medicine Physicians
Dr. Sherif El-Refael, MD, EBNM
Dr. Maha Khalil, MD

Pat. Name Mrs. Tahany Tharwat Nashed Agamy	Pat. ID: 180937
Ref. Doctor: د/وائل سمير	Date : 10/03/2026

REPORT

POST-CONTRAST MRI EXAMINATION OF THE BRAIN REVEALED

Technique:-

- Axial T1, T2 and FLAIR WIs
- Coronal T2 WIs
- Post-contrast scans

Findings:-

Known case of breast cancer.

Compared to the lat available study dated 29/12/2025 the current study revealed:

- Further regressive course of the previously noted left temporo-occipital fairly defined cortical and subcortical space occupying lesion, it's currently measuring 2.9x1.5mm in its maximum AP, TS and CC dimension as compared to 3.6x2.2x2.5mm at the previous study. It elicits intermediate to low T1 signal, heterogenous bright T2 / FLAIR signal with evidence of irregular marginal post contrast enhancement as well as non enhancing central area of necrosis / cystic degeneration.
- Also regressive course of the surrounding vasogenic edema (only noted at the occipital lobe in today's study).
- The forenoted lesion and the surrounding oedema exert mild mass effect in form of effacement of the overlying cortical sulci.
- No significant mass effect on the left lateral ventricle in today's study.
- No newly developed lesions.
- Capacious both lateral ventricles.

Pat. Name Mrs. Tahany Tharwat Nashed Agamy	Pat. ID: 180937
Ref. Doctor: د/وائل سمير	Date : 10/03/2026

- Bilateral fronto-parietal subcortical few subacute lacunar infarcts are noted.
- No midline shift.
- Normal posterior fossa structures.
- No definite calvarial or skull base osseous lesions are noted.

Side finding:-

- Left maxillary sinusitis.

Conclusion:

- ⚡ Known case of left temporo-occipital metastatic brain lesion showing regressive course regards its size and the surrounding edema as described.
- ⚡ No newly developed lesions.

Best regards,

Dr. Ramy Edward Asaad, MD

Dr. Trez Ayad.MSc

Scan
MASSARRA

NAME: تهاني ثروت ناشد عجمي Prof. Dr. وائل سمير مقار	ID: 1985528 DATE 30/12/2025
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MRI EXAMINATION OF THE BRAIN

Technique of examination:

- Axial T1, T2 & Flair WIs
- Sagittal T1 and Coronal T2 WIs.
- IV contrast was given .

MRI Findings:

Known case of breast cancer compared to previous MRI dated 16-11-2025:

- The known well defined , dural-based space occupying lesion at the left occipito-temporal region has reduced marginally in size now measuring 28x16x24mm compared to 36x22x25 mm at maximum axial and CC dimensions in the comparative study.
- The lesion still shows peripheral enhancement and central necrosis with surrounding vasogenic edema – although the edema has decreased to some extent. Still noted some mass effect on the left lateral ventricle.
- No new extra or intra axial lesions.
- No midline shift or transtentorial herniation.
- Otherwise, normal size shape & position of the cerebral ventricles.
- No gross vascular abnormality.
- Normal cerebellum, brain stem & cervico – medullary junction.
- Normal sellar region.
- Normal extra axial spaces.

Opinion:

- *Mild reduction in the size of the dural metastatic lesion as well as the surrounding edema.*
- *No newly developed lesions.*

Best regards;

Prof. Hassan Elkiki

Dr. Khaled Adel, FRCR



مركز الجيزة للرنجة
GIZA SCAN

أ.د / حسن الكيكي
أ.د / محمد عبد اللطيف
د.د / رانيا حمدي
د.د / أيمن عبد الوهاب

Name: تهاني ثروت ناشد عجمي	ID: 1985528
Prof Dr : Mohamed Ibrahim Orabi	Date 16/11/2025

MRI EXAMINATION OF THE BRAIN

Technique of examination:

- Axial T1, T2 & Flair WIs
- Sagittal T1 and Coronal T2 WIs.
- IV contrast was given .

MRI Findings:

Known case of breast cancer showing:

- There is a well defined , dural-based space occupying lesion at the left occipito-temporal region measuring about 3.6x2.2x2.5 cm at maximum axial and CC dimensions.
- It exhibits heterogeneous low T1 and high T2/FLAIR signal. It minor foci of internal low signal on SWI images. No internal diffusion restriction noted.
- It is seen associated with extensive vasogenic edema and mass effect in the form of effacement of the related sulci and compression of the ipsilateral ventricle.
- **The post-contrast images revealed heterogeneous enhancement**
- No midline shift or transtentorial herniation.
- Otherwise, normal size shape & position of the cerebral ventricles.
- No gross vascular abnormality.
- Normal cerebellum, brain stem & cervico – medullary junction.
- Normal sellar region.
- Normal extra axial spaces.

Opinion:

- **Intra-cranial left occipito-temporal dural-based neoplastic mass lesion with heterogeneous enhancement suggesting metastatic nature .**

Best Regards;
Prof. Hassan Elkiki
Dr. Nourhan El-Shrief Msc.

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

الاسم: /

تحية طيبة وبعد،،،

إياه المطلب المقدم من

قسم رقم طبي

نتشرف بأن نوضح فيما يلي تقريراً عن حالته :

تاريخ وساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد انه:

د. محمد حبيب لاسون

م. ك. س. - م. د. ع. - م. د. ع.

تحريراً في

الموقع: التاريخ: ٢٠٢٢/١٢/٢٥

الطبيب المعالج

يتمتع

Pat. Name: Tahany Tharwat Nashed Agamy	Pat. ID: 180937
Ref. Doctor: د.ائل مطر	Date : 20/09/2026

POST-CONTRAST MRI EXAMINATION OF THE BRAIN REVEALED

Technique:-

- o Axial T1, T2 and FLAIR WIs
- o Coronal T2 WIs
- o Post-contrast scans

Findings:-

Known case of breast cancer.

Compared to the last available study dated 17/7/2026 the current study revealed:

- Minimalregressive course of the previously noted left temporo-occipital fairly defined cortical and subcortical space occupying lesion, it's currently measuring 1.3 x 2.3 x 1.9 cm in its maximum AP, TS and CC dimension as compared to 1.6 x 2.4 x 2 cm at the previous study. It elicits intermediate to low T1 signal, heterogenous bright T2 / FLAIR signal with evidence of irregular post contrast enhancement as well as non enhancing central area of necrosis / cystic degeneration.
- Still noted moderate amount of surrounding vasogenic edema (still noted at the occipital lobe in today's study).
- The forenoted lesion and the surrounding edema exert mild mass effect in form of effacement of the overlying cortical sulci.
- No newly developed lesions.
- Mildly dilated both lateral ventricles.
- Bilateral fronto-parietal subcortical few subacute lacunar infarcts are noted.
- No midline shift.
- Normal posterior fossa structures.
- No definite calvarial or skull base osseous lesions are noted.

Conclusion:

- ✚ Known case of left temporo-occipital metastatic brain lesion showing minimal regressive course regards its size with rather stationary course of the surrounding edema as described.
- ✚ No newly developed lesions.

Best Regards,

Dr. George Ayad M.D

مركز خدمات الأشعة
الرئيسية التخصصية

4 شارع مسرة - دوفى الفرنج - القاهرة - 11511 - 11512 - 11513 - 11514 - 11515 - 11516 - 11517 - 11518 - 11519 - 11520 - 11521 - 11522 - 11523 - 11524 - 11525 - 11526 - 11527 - 11528 - 11529 - 11530 - 11531 - 11532 - 11533 - 11534 - 11535 - 11536 - 11537 - 11538 - 11539 - 11540 - 11541 - 11542 - 11543 - 11544 - 11545 - 11546 - 11547 - 11548 - 11549 - 11550 - 11551 - 11552 - 11553 - 11554 - 11555 - 11556 - 11557 - 11558 - 11559 - 11560 - 11561 - 11562 - 11563 - 11564 - 11565 - 11566 - 11567 - 11568 - 11569 - 11570 - 11571 - 11572 - 11573 - 11574 - 11575 - 11576 - 11577 - 11578 - 11579 - 11580 - 11581 - 11582 - 11583 - 11584 - 11585 - 11586 - 11587 - 11588 - 11589 - 11590 - 11591 - 11592 - 11593 - 11594 - 11595 - 11596 - 11597 - 11598 - 11599 - 11600 - 11601 - 11602 - 11603 - 11604 - 11605 - 11606 - 11607 - 11608 - 11609 - 11610 - 11611 - 11612 - 11613 - 11614 - 11615 - 11616 - 11617 - 11618 - 11619 - 11620 - 11621 - 11622 - 11623 - 11624 - 11625 - 11626 - 11627 - 11628 - 11629 - 11630 - 11631 - 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جدة شريفه في كبرها وكرمها
بطاقة تحقيق الشخصية

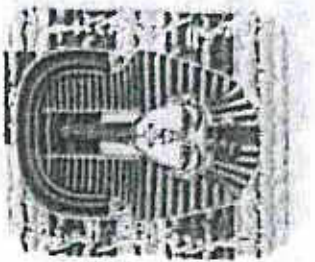


تهلتي
ثروت ناشد عجمي
٦ ش عمر مكرم - حي الزهور
العمرانية - الجيزة

١٩٧٢/٠٤/١٩

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KP1223106



٢٠٢٤/٠٦ ٢٧٢٠٤١٩٠١٠٣٤٢٧
حاصلة على بكالوريوس علوم حاسب آلي

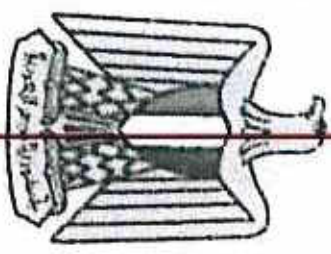
متروجة

مسيحية

انثى

هانى حنا شاكر عطالله

البطاقة سارية حتى ٢٠٣١/٠٥/٢١



جولف
روستاني
9-1



مبادرة رئيس الجمهورية
لحتم صحة المرأة المصرية

المملكة العربية
السعودية
1448

09/08/2026

MDT FORM

Name:	Tahani Tharwat Nashed Agamy		MRN:	9593
Age:	54 Y	ID:	27204190103427	
Menopausal Sta	postm	Phone:*	01222332820	
NRS SCORE:		Marital Status:		
BMI:		Family History:		
Gender		Reason of MDT:	last mdt for gemenzar herceptin no approval of herceptin	
CO Morbidity:		ECOG Performance Status	0 00 10 20 30 4	
Surgical History:*	left wle			

Clinical Data:

R

L



T N M

BX:	PR:	HER2:	KI67:
ER:			

HISTORY:

postm/lef metastatic breast cancer to brain er+ve pr+ve her23+
history
left breast cancer ct3 n3 no rec tchp on 5/11/2024 left wle ypt2 ypn1 rec
port kadcyla x12 last 8/2025
11/2025 brain mets rec wci str xeloda bykerb from 1/2026
pet ct on 13/7/2026: met progression with mild size regression left
occipital 2.3x1.6 suv 19.7
mri brain on 17/7/2026 mri progression of left temporal sol m 1.6x2.4x2
comp to 0.2x0.9x1.5
pt rec srs to left parietal lesion from 15/7 to 1/8 mdt for gemenzar herceptin
trial of enhertu from ngo
pt st gemenzar singler agent ct d1 5/9
but no approval of herceptin at MOH

Clinical Staging:

Pathological Staging:

Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

Continue On gemtrobline till approval of Enhertu in NGO

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

Blood Transfusion Monitoring Form

Patient name: Mr. Mohamed
MRN: 9593

Date of delivery: 6.19.2024 Time of delivery: 5:00 Transfusion start time: 6:30 End Time: 8:30
Blood Group & RH Type: (B+) Type of blood unit: (B+) Blood unit No: 2704

Blood unit double checking:	☐ Assigned nurse	Name & ID: <u>Mohamed 3059</u> Signature: <u>Mohamed</u>	☐ Charge Nurse	Name & ID: <u>ASTGA 3912</u> Signature: <u>3012</u>
Patient Identification double checking (bedside):	☐ Assigned nurse	Name & ID: <u>Mohamed 3059</u> Signature: <u>Mohamed</u>	☐ Charge Nurse	Name & ID: <u>ASTGA 3912</u> Signature: <u>3012</u>

Nursing Observation: ☒ No reaction ☐ Reaction (Rash, Fever, Shivering, Facial edema, Laryngeal Spasm, ...)
If reaction, Notified to: Time: ... : ... Documented in Blood Transfusion Reaction Form ☐ Yes ☐ No

	Time	Temp.	Pulse	SPO2	RR	BP	Pain
Baseline vital signs	6:30	36.5	85	100	16	105/73	0
After 5 minutes	6:35	36.7	86	100	16	110/75	0
After 10 minutes	6:40	36.6	88	99	17	117/80	0
After 15 minutes	6:45	36.6	87	99	18	122/83	0
After 30 minutes	7	36.8	90	98	18	125/84	0
After 45 minutes	7:15	36.8	92	98	17	130/85	0
After 60 minutes	7:30	36.7	93	100	20	132/87	0
After 90 minutes	8	37	95	99	19	130/85	0
After completion of transfusion	8:30	36.7	95	98	18	128/82	0
After 5 minutes from completion of transfusion	8:35	36.7	92	99	18	126/83	0
After 1 hour from completion of transfusion	9:30	37	93	99	17	125/84	0
After 2 hours from completion of transfusion	10:30	36.8	94	100	18	126/86	0

Nurse Name & ID: Mohamed 3059 Signature: Mohamed Date: 6.19.2024 Time: 8:30
said

* Use separate form for each unit transfused.

Diet Type: Vegetarian[illegible]

* All orders, modifications, discontinuations, and corrections should be dated, timed, and signed.

* Do NOT use abbreviations.

**This form is NOT intended for use for medications orders.*

الرقم الطبي:	9593
بهاش بروت باسد تحيين	
اسم المريض:	
تاريخ الميلاد:	19/04/1972
الرقم الطبي:	1547/2024

Blood /Blood Components Transfusion Coi.....
اقرار بالموافقة على نقل الدم / مشتقاته

٢٥٤٠٠١٩٠١٠٢٥٢٥١

رقم قومي / جواز سفر

٩٥٩٣

٩٥٩٣

اقرار الموقع ادناه (المريض / ممثل القانوني)

الرقم الطبي

ان قد وافقت على اعطاء الدم و/او احد مشتقاته وذلك بعد ان قام الطبيب بشرح ما يلي بشكل واضح ومفهوم:

١- سبب الحاجة الى نقل الدم في حالات:

ا- نقل خلايا الدم الحمراء: في حالة معاناة الشخص من فقر الدم الذي غالبا ما يحدث بسبب نقص الحديد او المعاناة من اضطراب الدم.

ب- نقل الصفائح الدموية: لتعويض المفقود لدى الأشخاص الذين يعانون من انخفاض في عدد الصفائح الدموية نتيجة العلاج الكيميائي أو اضطراب

الصفائح الدموية

ج- نقل البلازما: تحتوي البلازما على بروتينات مهمة لصحة الجسم وغالبا ما يتم نقلها للأشخاص في حالة المعاناة من الحروق الشديدة أو الالتئابات أو

فصل الكبد.

٢- الفوائد المتوقعة من هذا الاجراء

٣- المخاطر والمضاعفات المحتملة بما في ذلك:

ا-ردود الفعل التحسسية والتي قد تشمل قشعريرة، حمى، أو تورم.

ب-ارتفاع درجة الحرارة.

ج-المدوى النادرة بأمراض فيروسية وبكتيرية.

د-مشاكل في القلب أو الرئة نتيجة فائض السوائل أو التحسس الشديد، تفاعل مع خطأ في المطابقة أعراق الدم، مما قد يؤدي إلى تفاعل جسدي شديد.

هـ-حدوث فرط الحديد: قد يتسبب فرط الحديد بتلف في مناطق وأعضاء مختلفة من الجسم إن لم يتم تداركه وعلاجه في الوقت المناسب، لا سيما إن

حدث في القلب أو الكبد.

و- إصابة لمشاكلات صحية في الرئتين، مثل: إصابة الرئة المتعلقة بنقل الدم، وهو مرض نادر قد يتسبب بتلف الرئة والوفاة.

٤- البدائل الممكنة لنقل الدم إن وجدت.

٥- حقني في طرح الأسئلة والحصول على الإجابات الكاملة قبل اتخاذ القرار.

اقرار الموقع ادناه بانني على كامل المعرفة والفهم لكل مشتقات هذا الاقرار واتحمل المسؤولية كاملة عن كل ما فيه

اسم المريض (او ممثله القانوني):

٩٥٩٣

اسم الطبيب المعالج / الرقم الوظيفي:

الدرجة القرنية:

رقم قومي / جواز سفر:

التوقيع:

٩٥٩٣

التاريخ / الوقت:

اسم الشاهد:

التاريخ / الوقت:

رقم قومي / جواز سفر:

التوقيع:

Blood / Blood Components Request

Patient Name:
MRN:

Room no: 1158.C	Sex: ☐ Male ☒ Female	Age: 55Y
Diagnosis: Hx B-1		
Indication for transfusion:		
History of Previous Blood Transfusion: ☐ No ☒ Yes		History of Previous Blood Transfusion Reaction: ☐ No ☒ Yes, specify
History of Previous Blood Transfusion Reaction: ☐ No ☒ Yes		
Number of Units Needed:		To be delivered to:
☐ Whole Blood..... Units 500 cc ☐ Packed Cells..... Units ☐ Platelets..... Units ☐ F.F.P..... Units ☐ Cryoprecipitate..... Units ☐ Auto transfusion..... Units ☐ Other: Units		☐ OR ☐ ED ☐ ICU ☐ BMT
Urgency:		Frequency of Transfusion:
☐ Immediate: Needed in less than 20 minutes (Without compatibility testing). ☐ Urgent: Cross matching tests take 1 hour ☐ Routine: compatibility test take two hours		
Requesting Physician Name & ID:	Signature: R. El /	Date: 9/10
Sample taken by (nurse) Name & ID:	Signature: M. /	Date: 9/10

For Blood Bank:

Date	Blood Group	Bag no.	Time	Name & ID	Signature
5-9	Bp2	1224	6:26	Princess	5-9

Blood Delivery form

2567	نمبري قروب لظف
MRN:	1547/24

نقطة دولة نقدي

Components	Amount	Order Status		Time of Order
☐ Whole blood		☐ Scheduled		
☐ Packed RBCs	One Unit	☐ Emergency		
☐ Filtered Packed RBCs		☐ High Emergency (without cross matching)		
☐ Irradiated Packed RBCs		☐		
☐ Fresh Frozen Plasma		☐		
☐ Platelet concentrate		☐		
☐ Cryo precipitate		☐		
☐ Irradiated Platelet		☐		

Time of Delivery of Blood Components:

6:25:26 p

Blood Group:

(B)

RH Group (D antigen):

Positive

Antibody Screening:

Cross Matching:

Matched

Bag No.:	2724	23944933		
Donation time	25/8/2026	EXPIRATION DATE	29/9/2026	
Donation time		EXPIRATION DATE		

Blood Bank Physician Name & ID	Signature	Date of Delivery	Time
NYRAA SAYED	NYRAA SAYED	5-2026 سبتمبر	6:25 p

Assigned Nurse/ Healthcare Assistant Name & ID	Signature	Date of Delivery	Time
		5-2026 سبتمبر	6:25 p